

Adult Sepsis Screening and Immediate Action Tool

Complete and file in medical record



Kettering General Hospital **NHS**
NHS Foundation Trust

Caring at its best

Name:

Date of Birth:

Hospital Number:

Affix Hospital Label if available

1. At least one of the following present?

- ☐ Early Warning Score 3 or more **OR**
☐ Patient looks unwell **OR**
☐ Concern regarding acute change in mental state

Y

2. Is the clinical picture suggestive of an infection?

If there is a high probability of a non-infective explanation for clinical features (eg AMI, PE, liver failure, pancreatitis or stroke) then manage as low risk of sepsis

- ☐ Chest
☐ Urinary Tract
☐ Cellulitis, necrotizing fasciitis
☐ Abdominal
☐ Bone or joint
☐ Meningitis
☐ Device related (eg catheter, line)
☐ Other, state:
☐ Yes, but source unclear

Y

3. At least one red flag present?

- Assessment **MUST** allow for patients usual chronic baseline
- Obstetric patients: use corresponding red MEOWS triggers

- A** ☐ Respiratory rate 25/min or more
B ☐ New need for >40% O₂ to keep saturations over 91% (saturations > 87% in COPD)
☐ Systolic BP < 91mmHg or fall of 40 from normal
C ☐ HR >130/min
☐ No urine output for 16hrs or UO<10ml/hr
D ☐ New onset delirium
☐ Responds only to voice or pain / unresponsive
E ☐ Non-blanching rash/ mottled / ashen / cyanotic
☐ Neutropenia or chemotherapy within last 6 weeks

Low Risk of SEPSIS

Treat to local protocols, review if patient deteriorates. Consider other diagnoses.

Moderate Risk of SEPSIS

Sepsis Likely / Present

- Inform responsible clinician
- Consider Sepsis Six interventions (see overleaf)
- Begin at least hourly observations
- Act on early warning score triggers
- Send appropriate microbiological samples (including blood culture)
- Send blood samples for FBC, CRP, U&E, LFT coagulation, Blood gas (venous or arterial) Glucose, ensure results are reviewed
- Source specific antimicrobial prescribing based on local policy (e.g. 4hr CAP bundle)

If EWS 4 or more **AND** Lactate ≥ 2 or AKI ≥ 2
If YES Then treat as RED FLAG SEPSIS

Time zero = Time of abnormal blood results availability

HIGH Risk of SEPSIS

Red Flag Sepsis

This is a time critical condition, immediate action is required!

Start Sepsis 6 bundle NOW (see overleaf)

- ☐ Inform resident senior doctor
☐ Inform outreach team (ward patients):
BLEEP LRI 5293 / GH 2808 / LGH 3457
☐ Inform Sepsis Team (ED): **CALL # 6826**

Time Zero: (ED, time of admission. Ward, time of first red flag)

Target Time: (Time when sepsis six to be complete. Time zero plus 1 hr)

Delivery of Sepsis Six by junior staff must not be delayed. Resident senior doctor review can stop the process on the following grounds:

- ☐ Patient is End of Life
☐ Patient low suspicion of infection
☐ Red Flag due to chronic disease

STOP

SURNAME & GRADE

SIGNATURE

DATE & TIME

PRINT NAME:

Title:

Date:

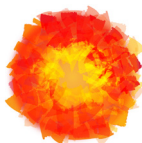
Time:

Sepsis Six Bundle

Complete in one hour.

Actions should be carried out simultaneously.

Use sepsis box / pack to support delivery of sepsis six



THE UK
SEPSIS
TRUST

Supporting Resources



Sepsis
Frequently Asked Questions



How to:
Take a blood culture
Draw up meropenem
Use a sepsis box

1	Administer supplementary oxygen (if required) - Aim to keep saturations > 94% - COPD: Adjust target saturations to 88-92%	Time Started	Name	Reason not administered
2	Blood Culture & Source Management - Take blood cultures (before IV antibiotic) - Think source confirmation and control! - Consider also sputum, urine, CSF, line culture/removal - involve appropriate surgical team / radiologist as indicated - For Community Acquired Pneumonia start 4 hr CAP Bundle	Time Taken	Name	Reason not taken
3	Give IV antibiotics PRESCRIBE STAT (TIMED). GIVE YOURSELF OR MAKE SURE SOMEONE DOES Red Flag Sepsis: Meropenem IV 1g stat (+/- second dose at 8hrs) and review at first inpatient consultant assessment (microbiology advice may be needed at this stage) - Sepsis: According to local antimicrobial policy	Time Given	Name	Reason for departure from prescribing guidance
4	Give a fluid challenge Check and monitor response If SBP <90mmHg or Lactate >2 - Give 500mls Hartmann's or 0.9% NaCl over 15 mins, repeat once if necessary - Senior resident doctor review to exclude other causes of shock before giving up to 30 ml/kg If SBP >90mmHg and Lactate <2 consider IV fluids	Time Given	Name	Reason not given
5	Measure lactate - Obtain blood gas - venous or arterial - If lactate >4mmol/L refer to critical care - Ensure samples are sent for FBC, CRP, U+E, LFT, coag screen - Repeat lactate after fluid challenge	Time Taken	Name	Reason not done
6	Measure urine output - Ensure hourly fluid balance chart commenced - Catheterise if AKI / SBP <90 / Lactate >2 - Monitor Vital Signs at 15-30mins intervals until EWS below 3	Time Started	Name	Reason not started
Escalation	Critical Care Medical Team refer if patient: - SBP <90 and lactate >2 after fluid resuscitation - Has Red Flag Sepsis and lactate >4 - Has Red Flag Sepsis and requires >50% O₂ - Has Red Flag Sepsis and significant respiratory/ cardiovascular/ CNS or renal dysfunction.	Time Referred	Name of Referrer	Reason <u>NOT</u> Referred:
			Name of ICU Doctor	